

Triage today depends on which nurse is at the desk.

- Same chest pain walks in twice → two different acuity scores, two different waits.
- No paper trail of why a tier was assigned.
- No flag when a clock is about to be missed (door-to-ECG, last monitored bed).
- When something goes wrong, the chart doesn't show what was considered or ruled out.

The first 10 minutes of an ED visit are the most consequential — and the least documented.

Side-by-side input and output. Cited. Overridable.

INPUT (left column)

- Vitals, chief complaint, nurse notes
- History, allergies, meds, prior visits
- Arrival timeline
- Add-update form → re-runs assessment

OUTPUT (right column)

- Urgency badge + confidence
- Constraints: beds, staff, time-target breaches
- Hypotheses with confidence bars
- Next actions  Immediate ·  Monitor ·  Escalate
- Explanation cites real ESI rule_ids
- Follow-up checklist (owner + deadline)
-  Review note: why this tier, what would flip it

Below the columns: clinician confirms or overrides — every override logged with a reason.